

Session 2 — FIND & READ: Solutions and facilitation notes

Foundations of Clinical Research — Level 1 · Neudata · *Instructor copy — do not hand out before the exercises*

These are **model** answers. Many other answers are acceptable: reward work that is specific, consistent and could be acted on. Use the "what good looks like" boxes when you circulate.

Recap quiz — Session 1: answers

1. **Audit** — checking practice against an agreed standard.
2. **P** ICU staff · **I** hand-hygiene feedback · **C** no feedback · **O** hand-hygiene compliance · **T** 3 months.
3. **Feasible** — a mortality outcome needs more than 1,000 patients.
4. **"To estimate the proportion with delirium"** — a measurable verb; "understand" is not measurable.
5. **H0**: ICU length of stay is the same with early mobilisation and with usual care.

If many miss 4 or 5, spend one extra minute: objectives use measurable verbs; H0 always states "no difference".

Check your understanding (segment 2.1) — AND, OR or NOT?

1. **AND**. 2. **OR**. 3. **Fewer** (AND narrows). 4. It removes every paper that mentions children anywhere — including good adult studies that mention children once.

Check your understanding (segment 2.2) — OK or not OK?

1. **Not OK** — patchwriting is plagiarism even with a citation. 2. **OK** (but rare in medical writing). 3. **Not OK** — someone else's finding needs a citation. 4. **Not OK** — you may repeat an error; read and cite the original.

Check your understanding (segment 2.3) — spot the spin

1. Causal word for an observational design → *"was associated with lower pneumonia"*.
2. $p = 0.31$ is no evidence of a difference → *"no significant difference; the confidence interval was wide"*.
3. Headline is a secondary outcome → lead with the primary result.
4. Before–after change without a comparison group → *"mortality was lower after implementation; without a control group we cannot attribute this to the protocol"*.

Exercise 1 — Boolean search builder

Warm-up: septic shock OR fluids gives more hits (either term). sepsis gives more hits than sepsis NOT neonatal (NOT removes papers).

Model string for the running example (as in the demo):

```
("Shock, Septic"[Mesh] OR "septic shock"[tiab] OR sepsis[tiab])
AND ("Fluid Therapy"[Mesh] OR "fluid restrict*"[tiab] OR "restrictive fluid*"[tiab] OR "conservative
AND ("Mortality"[Mesh] OR mortality[tiab] OR death[tiab])
```

Sensible extra synonyms for the I block: de-resuscitation[tiab], deresuscitation[tiab], "fluid balance"[tiab], "Water-Electrolyte Balance"[Mesh].

Sensible filters (each with a reason): article type *Randomized Controlled Trial + Systematic Review* (treatment question); *Adult: 19+ years* (our population); publication date *10 years* (current practice). Avoid *Free full text* (loses key papers).

Model string for Scenario C (delirium)

```
(Delirium[Mesh] OR delirium[tiab] OR "acute confusion"[tiab])
AND ("Hip Fractures"[Mesh] OR "hip fracture*"[tiab])
AND (prevent*[tiab] OR "multicomponent"[tiab] OR "non-pharmacological"[tiab] OR bundle[tiab])
```

Common faults

- Missing brackets: A OR B AND C is read differently from (A OR B) AND C.
- AND inside a block (septic shock AND sepsis) — synonyms need OR.
- A block for the comparison ("usual care") — adds nothing and loses papers.
- NOT used to remove a topic the paper only mentions in passing.
- Search words in the local language: PubMed indexes English titles and abstracts.

Exercise 2 — Paraphrasing: model answers

Part A

Version	Verdict	Why
a)	Problem — patchwriting	Same structure and almost the same words as the source; one word swapped ("mortality" → "death"). Plagiarism even though it is cited.
b)	OK	New structure, adds context (matched registry cohort, 28-day mortality), keeps the calibrated word "associated", gives numbers and the citation.
c)	Problem — distorts the source	"Giving more fluid ... doubles the risk" turns an observational <i>association</i> into causation, and "doubles" overstates (HR 2.13 only for day 2; 1.56 for day 3). Citation does not fix misrepresentation.
d)	Problem — no citation	Good wording, but presenting someone else's finding without a citation is plagiarism.

Part B — model paraphrase

"Mortality and days alive without organ support at day 28 did not differ between the two groups, although the pilot trial was too small to detect such differences [1]."

What good looks like: new structure; facts unchanged (28 days, mortality, days alive without life support, similar); no stronger claim (do **not** write "restriction is safe"); citation. Adding "the trial was too small" is accurate context from the paper (48 patients; not powered for mortality).

Exercise 3 — Paper 1's abstract: answer key

Item	Answer (from the abstract)
Design (and blinding)	Multicentre, randomised, controlled pilot trial; single-blind — patients and statisticians blinded, clinicians not
P	Adults 18–85 years with septic shock, in the first 24 h of vasopressor infusion, in three ICUs; excluded: AKI needing renal replacement, end-stage CKD, severe malnutrition
I vs C	Optimised fluid restriction (reducing maintenance fluids and fluids for drug dilution during the first 7 days) vs standard fluid strategy
Primary outcome	Cumulative fluid balance (ml/kg) in the first 5 days — not mortality
Numbers	1,201 screened; 50 randomised; 2 withdrew (control); 48 analysed (24 per group)
Main result	Median cumulative balance at day 5: 6.9 vs 35.0 ml/kg; absolute difference 13.2 ml/kg (95% CI –15.2 to 41.6), $p = 0.42$
Conclusion	"...did not reduce the overall fluid balance at day 5"
Matches the result?	Yes — the CI includes zero; the conclusion says "did not reduce"
Spin in the abstract?	No in the abstract conclusion. (Spin appears in the <i>Discussion</i> , which calls $p = 0.42$ a "non-significant trend".) Watch-point: the fluid <i>volumes</i> (89.7 vs 114.3 ml/kg, $p = 0.0506$) are a secondary result, and the abstract reports four decimals for a borderline p — mild emphasis, not a claim.
Registration	ClinicalTrials.gov NCT04947904 (registered 1 July 2021, before recruitment began in September 2021)
Answers our 28-day mortality question?	No . It is a small pilot powered for fluid balance; 28-day mortality was a secondary outcome and was similar between groups — "no evidence of a difference", not "no difference"

Fast finishers (Paper 2): the title says "**Impact**" (causal); the abstract conclusion correctly says "**associated**".

Common errors to correct in the debrief

- Writing "mortality" as the primary outcome.
- Copying only the p -value and not the estimate with its 95% CI.

- Calling the trial "double-blind" — clinicians, who prescribe the fluids, were not blinded.
- Reading the medians (6.9 vs 35.0) as "five times less fluid balance, so it works" — the CI crosses zero.

Exercise 4 — Proposal builder 2: what good looks like

Continuing the worked model from Session 1 (the feasible rewrite of the running example).

Search (documented): PubMed, date, the string from Exercise 1, filters (article type RCT + systematic review; adults; 10 years), number of hits; plus a check of ClinicalTrials.gov for ongoing trials.

Working title: *Restrictive versus usual fluids in adults with septic shock: a pilot randomised trial* (13 words).

Keywords: Fluid Therapy; Water-Electrolyte Balance; Critical Care; Vasoconstrictor Agents (MeSH terms not already in the title).

Problem statement (≈ 300 words, three paragraphs)

1. *Known:* septic shock is a common cause of ICU death; early fluid resuscitation restores perfusion, but continued fluid often produces a positive fluid balance, which observational studies associate with higher mortality (e.g. Hyun 2023).
2. *Not known:* large trials of restrictive fluids (CLASSIC 2022; CLOVERS 2023) found no mortality difference, and a pilot targeting hidden "non-resuscitative" fluids (Boulet 2024) did not significantly reduce fluid balance at day 5. Whether a restrictive protocol can reduce fluid balance in *our* setting is unknown.
3. *Why here:* a 3-month audit in our ICU found that most septic shock patients were fluid-positive by day 3 (state the number from your own audit), and practice varies between consultants.

Five key references (model)

#	Reference	Why included
1	Boulet N, et al. <i>Crit Care</i> 2024;28:429 — pilot RCT, n = 48	Closest design and outcome; primary outcome not significantly different
2	Hyun DG, et al. <i>Ann Intensive Care</i> 2023;13:105 — matched cohort	Positive balance on days 2–3 associated with higher 28-day mortality
3	Meyhoff TS, et al. (CLASSIC) <i>N Engl J Med</i> 2022;386:2459–70	Large RCT, septic shock, no difference in 90-day mortality
4	Shapiro NI, et al. (CLOVERS) <i>N Engl J Med</i> 2023;388:499–510	Large RCT, sepsis-induced hypotension, restrictive vs liberal, no difference in 90-day death before discharge
5	Evans L, et al. Surviving Sepsis Campaign guidelines 2021. <i>Crit Care Med</i> 2021;49:e1063–e1143	Current guideline; shows the remaining uncertainty

Instructor note: check citation details against PubMed before distributing; groups must verify every reference themselves (practise exactly that habit).

Marking guide for circulating (not a grade)

Element	Good	Needs work
Search	String, database, date, filters and hits recorded; brackets correct	"We googled it"; no date; AND inside a block
References	5, relevant, include the best available design, exported to a reference manager	Websites, textbooks only, typed by hand, unread
Literature matrix	Same columns for every paper; limitation column filled	Copied abstract sentences
Problem statement	Known → gap → local reason, each fact cited, ends with the question	A literature dump, or no local reason
Title & keywords	10–16 words; P, I/E, C and design visible; calibrated words; keywords add MeSH not in the title	"Fluids in the ICU"; abbreviations; "impact of" for an observational design
Integrity	Own words, every claim cited	Sentences pasted from abstracts

Exit question — answers

1. **OR** gives more hits.
2. Paper 1's primary outcome was **cumulative fluid balance at day 5 (ml/kg)** — not mortality.